



SURGICAL PATHOLOGY REPORT

Patient Name:

Med. Rec. #:

DOB:

Gender:

Ref. Physician:

Patient Address:

Ref. Source:

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Billing Type:

Additional Copy to:

INPATIENT

Clinical Diagnosis & History:

ear old male with recurrent T1HG, Tis despite BCG.

Specimens Submitted:

- 1: Left distal ureter; excision
- 2: Lymph nodes, left pelvic; dissection
- 3: Left ureter #2; excision
- 4: Right ureter; excision
- 5: Bladder, prostate, seminal vesicles, urethra; cystoprostatectomy
- 6: Lymph nodes, right common iliac; dissection
- 7: Lymph nodes, right pelvic; dissection
- 8: Lymph nodes, right hypo gastric; dissection
- 9: Lymph nodes, left common iliac; dissection
- 10: Lymph node, left pelvic # 2; biopsy
- 11: Lymph node, left hypo gastric; biopsy
- 12: Right vas deferens; excision
- 13: Left vas deferens; excision

ICD-O-3
Carcinoma, urothelial
8/26/13
Site ^{C67} Bladder, anterior wall
C67.3
path ^{C67} Bladder, NOS
C67.9
JW 6/18/13

DIAGNOSIS:

1. **Left distal ureter; excision:**
 - Urothelial carcinoma in situ.
2. **Lymph nodes, left pelvic; dissection:**
 - Lymph Node Dissection:
 - Benign lymph nodes
 - Number of lymph nodes examined: 5
3. **Left ureter #2; excision:**
 - Urothelial carcinoma in situ, best seen on actual frozen section.
4. **Right ureter; excision:**
 - Benign segment of ureter.
5. **Bladder, prostate, seminal vesicles, urethra; cystoprostatectomy:**
 - Tumor Type:
 - Invasive urothelial carcinoma, NOS
 - Histologic Grade:
 - High grade
 - Pattern of growth of the Non-Invasive component:
 - Flat (in situ carcinoma)
 - An inverted non-invasive component is also present but CIS predominates

SURGICAL PATHOLOGY REPORT

Pattern of growth of the Invasive component:
Infiltrating

Tumor Multicentricity:
Identified
Multiple foci of invasive disease are present but the lesion in the left anterior wall is the most invasive, extending into perivesicle fat

Bladder Local Invasion:
Perivesical soft tissues

Extravesical Tumor Extension:
Right ureter uninvolved
Left ureter involved
Urethra uninvolved
The intramural right ureter contains carcinoma

Vascular Invasion:
Identified
LVI is multifocal

Perineural Invasion:
Identified

Surgical Margins:
Tumor present at left ureteral margin
See specimen number 3 for true left ureteral margin

Non-Neoplastic Mucosa:
Exhibiting chronic cystitis
Exhibiting granulomatous cystitis
Exhibiting foreign body reaction
Exhibiting proliferative cystitis (Brunn's nests, cystitis cystica, cystitis glandularis)

Prostate:
Nodular hyperplasia
Other The prostatic urethral is involved by urothelial carcinoma with underlying focal superficial stroma invasion

Seminal Vesicles:
Not involved

Perivesical Lymph Nodes:
Metastatic carcinoma present in 2 of 3 nodes

The Pathologic Stage is (AJCC 2002):
pT3 (Invasion of perivesicle soft tissue)

PR1 (Prostatic stromal invasion)
Comment: The nodal metastases consist of isolated cells or small clusters of cells present within the peripheral sinus without evidence of extranodal extension

6. Lymph nodes, right common iliac; dissection:

Lymph Node Dissection:
Benign lymph nodes
Number of lymph nodes examined: 8

7. Lymph nodes, right pelvic; dissection:

Lymph Node Dissection:
Benign lymph nodes
Number of lymph nodes examined: 8

SURGICAL PATHOLOGY REPORT

Necrotizing granulomatous inflammation consistent with BCG treatment

8. Lymph nodes, right hypo gastric; dissection:

Lymph Node Dissection:

Benign lymph nodes

Number of lymph nodes examined: 4

9. Lymph nodes, left common iliac; dissection:

Lymph Node Dissection:

Benign lymph nodes

Number of lymph nodes examined: 10

10. Lymph node, left pelvic # 2; biopsy:

Lymph Node Dissection:

Benign lymph nodes

Number of lymph nodes examined: 2

11. Lymph node, left hypo gastric; biopsy:

Lymph Node Dissection:

Benign lymph nodes

Number of lymph nodes examined: 1

12. Right vas deferens; excision:

- Benign segment of vas deferens

13. Left vas deferens; excision:

- Benign segment of vas deferens

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Gross Description:

1)The specimen is received fresh, labeled "Left distal ureter". It consists of a 0.3 x 0.3 x 0.2 cm segment of ureter. Entirely frozen.

Summary of sections

FSC- frozen section control

2)The specimen is received fresh, labeled "Left pelvic lymph nodes". It consists of five lymph nodes ranging from 0.3 to 2.5 cm. Entirely frozen.

Summary of sections

FSCA- frozen section control, one lymph node, bisected

FSCB- frozen section control, one lymph node, bisected

FSCC- frozen section control, three lymph nodes

SURGICAL PATHOLOGY REPORT

3) The specimen is received fresh, labeled "Left ureter #2". It consists of a 0.3 x 0.3 cm fragment of mucosa with attached 0.4 cm adipose tissue. The mucosa is entirely frozen.

Summary of sections
FSC- frozen section control

4) The specimen is received fresh, labeled "Right ureter". It consists of a 0.5 x 0.4 x 0.3 cm segment of ureter. Entirely frozen.

Summary of sections
FSC- frozen section control

5. The specimen is received fresh labeled, "Cystoprostatectomy". It consists of a bladder or prostate and seminal vesicle specimen measuring 13 cm from superior to inferior, 13.5 cm laterally and 5.5 cm from anterior to posterior. The specimen is inked as follows: The right side is inked green and the left side is inked blue. The prostatic urethral margin is shaved and submitted for frozen section. The bladder measures 9.5 x 9.3 x 4.5 and is opened along the anterior midline to reveal an irregular ulcerating lesion measuring 2 x 1.5 cm which is grossly situated within the left anterior wall. The lesion is situated 1 cm from the left orifice and 3 cm from the right. Both ureters are probe patent measuring up to 0.2 cm in maximum diameter. The lesion is sectioned to reveal possible extension into muscularis. Extension to the inked fat is grossly not identified. A second irregular ulcerating lesion measuring 2.6 x 1.5 cm is seen 2.7 cm from the first tumor and grossly situated within the right anterior / lateral wall. The lesion is present within 0.3 cm from the right orifice and 2.5 cm from the left. Both ureters are probe patent measuring up to 0.2 cm in maximum diameter. The lesion is sectioned and appears limited to lamina propria. Extension to the inked fat is grossly not identified. A third tan polypoid nodule is present at the dome of the bladder measuring 0.9 x 0.9 cm and 4.5 cm from the first lesion. A fourth possible lesion is identified also in the dome about 1 cm from the nodule. The remaining bladder mucosa is grossly unremarkable. Discrete perivesical lymph nodes are grossly not identified. The prostate measures 3.5 x 2.5 x 2 cm and is serially sectioned to reveal unremarkable fibromuscular tissue. Representative section of the specimen, including entire lesions are submitted including two sections from each prostatic quadrant and transition zone. TPS is submitted. The specimen is submitted for lymph node dissection and all possible lymph nodes are submitted.

Summary of sections:
FSC - frozen control (urethral margin)
RUM - right ureter margin
LUM - left ureter margin
L1 - lesion in left anterior wall
L2- lesion in right anterior/ lateral wall
N- nodule in dome
L4- fourth possible lesion
RUO - right ureter orifice
LUO - left ureter orifice
LP - left posterior wall
RP - right posterior wall (right and left anterior walls are almost entirely submitted with lesions)
TRI -- trigone
DOM - dome
F - perivesical fat
RSV - right seminal vesicle
LSV - left seminal vesicle
RAP - right apex prostate
LAP - left apex prostate
RAM - right anterior mid prostate
RPM - right posterior mid prostate
LAM - left anterior mid prostate
LPM - left posterior mid prostate
RAB - right anterior base prostate
RPB - right posterior base prostate

SURGICAL PATHOLOGY REPORT

LAB - left anterior base prostate
LPB - left posterior base prostate

6). The specimen is received fresh, labeled "Right common iliac lymph node" and consists of two tan firm lymph nodes measuring 1.3 x 0.8 x 0.3 and 1.5 x 0.8 x 0.4 cm. The lymph nodes are bisected and entirely submitted.

Summary of sections:

BLN - lymph nodes
PLN--possible lymph nodes

7). The specimen is received fresh, labeled "Right pelvic lymph nodes" and consists of multiple pink tan firm lymph nodes ranging from 0.3 to 3.1 cm in greatest dimension. All identified lymph nodes are submitted.

Summary of sections:

LN - lymph nodes
BLN - bisected lymph node
S1--serially sectioned lymph node
S2--serially sectioned lymph node

8). The specimen is received fresh, labeled "Right hypogastric lymph node" and consists of multiple pink tan firm lymph nodes ranging from 0.2 to 0.4 cm in greatest dimension. All identified lymph nodes are submitted.

Summary of sections:

LN - lymph nodes
PLB--possible lymph node

9). The specimen is received fresh, labeled "Left common iliac lymph nodes" and consists of multiple pink tan firm lymph nodes ranging from 0.2 to 2.2 cm in greatest dimension. All identified lymph nodes are submitted.

Summary of sections:

LN - lymph nodes
BLN - bisected lymph nodes

10). The specimen is received fresh, labeled "Left pelvic lymph node # 2" and consists of a firm tan lymph node measuring 3.5 x 2.0 x 1.0 cm. The lymph node is serially sectioned and entirely submitted.

Summary of sections:

SL--serially sectioned lymph node
PLN--possible lymph nodes

11). The specimen is received fresh, labeled "Left hypogastric node" and consists of a firm tan lymph node measuring 0.6 x 0.5 x 0.3 cm. The lymph node is entirely submitted.

Summary of sections:

LN - lymph node

SURGICAL PATHOLOGY REPORT

12). The specimen is received in formalin, labeled, "right vas deferens" and consists of two cylindrical portions of tan-white soft tissue averaging 2.5 x 0.4 cm. Sectioning reveals tan-white, pinpoint lumens. The specimen is entirely submitted.

Summary of sections:
U -- undesignated

13). The specimen is received in formalin, labeled, "left vas deferens" and consists of a single cylindrical portion of tan-white soft tissue averaging 5 x 0.3 cm with attached tan-purple, membranous tissue. Sectioning reveals a tan-white, pinpoint lumen. The specimen is entirely submitted.

Summary of sections:
U -- undesignated

Summary of Sections:

Part 1: Left distal ureter; excision

Block	Sect. Site	PCs
1	fsc	1

Part 2: Lymph nodes, left pelvic; dissection

Block	Sect. Site	PCs
1	fsca	1
1	fscb	1
1	fsc	1

Part 3: Left ureter #2; excision

Block	Sect. Site	PCs
1	fsc	1

Part 4: Right ureter; excision

Block	Sect. Site	PCs
1	fsc	1

Part 5: Bladder, prostate, seminal vesicles, urethra; cystoprostatectomy

Block	Sect. Site	PCs
1	BLN	2
1	DOM	1
1	F	1
1	fsc	1
5	L1	5
4	L2	4
2	L4	2
1	LAB	1
1	LAM	1
1	LAP	1
1	LN	4
1	LP	1
1	LPB	1

SURGICAL PATHOLOGY REPORT

1	LPM	1
1	LSV	1
1	LUM	1
1	LUO	1
2	N	2
1	RAB	1
1	RAM	1
1	RAP	1
1	RP	1
1	RPB	1
1	RPM	1
1	RSV	1
1	RUM	1
1	RUO	1
1	TRI	1

Part 6: Lymph nodes, right common iliac; dissection

Block	Sect. Site	PCs
2	BLN	4
1	PLN	3

Part 7: Lymph nodes, right pelvic; dissection

Block	Sect. Site	PCs
1	BLN	2
2	LN	5
3	S1	6
3	S2	6

Part 8: Lymph nodes, right hypo gastric; dissection

Block	Sect. Site	PCs
1	LN	4
1	PLN	1

Part 9: Lymph nodes, left common iliac; dissection

Block	Sect. Site	PCs
1	BLN	2
3	LN	8
1	PLN	1

Part 10: Lymph node, left pelvic # 2; biopsy

Block	Sect. Site	PCs
1	PLN	2
4	SL	8

Part 11: Lymph node, left hypo gastric; biopsy

Block	Sect. Site	PCs
1	LN	1

Part 12: Right vas deferens; excision

Block	Sect. Site	PCs
3	U	30

Part 13: Left vas deferens; excision

SURGICAL PATHOLOGY REPORT

Block 3 Sect. Site U PCs 15

Intraoperative Consultation:

1. Frozen section diagnosis: Left distal ureter (fs): Carcinoma in situ ()
Permanent diagnosis: same
2. Frozen section diagnosis: Left pelvic lymph nodes (fs): Benign lymph nodes (0/5) ()
Permanent diagnosis: same
3. Frozen section diagnosis: Left ureter #2 (fs): Carcinoma in situ ()
Permanent diagnosis: same
4. Frozen section diagnosis: Right ureter (fs): Benign ()
Permanent diagnosis: same
5. Frozen section diagnosis: Bladder, prostate, seminal vesicles urethra (fs): Benign urethral margin ()
Permanent diagnosis: same

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Tumor		☒
Case Is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	BTH Date Reviewed: 4/26/13	